

PROTECT YOURSELF FROM OCCUPATIONAL INFECTION

Give urgent attention to the health worker with a sharps injury or splash to eye, mouth, nose or broken skin with exposure to any of:

- Blood
- Blood-stained fluid/tissue
- Wound secretions
- Pleural/pericardial/peritoneal/amniotic/synovial/cerebrospinal fluid
- Vaginal secretions
- Semen
- Breast milk

Management:

- If broken skin, clean area immediately with soap and water.
- If splash to eye, mouth or nose, immediately rinse mouth/nose or irrigate eye thoroughly with water.
- Assess need for HIV and hepatitis B post-exposure prophylaxis ➤ 106.

Adopt measures to diminish your risk of occupational infection

Protect yourself

Adopt standard precautions with every patient:

- Wash hands with soap/water or use alcohol-based cleaner after contact with patients or body fluid.
- Dispose of sharps correctly in sharps bins.

Wear protective gear:

- Wear gloves when handling blood, body fluids, secretions or non-intact skin.
- Wear surgical mask if in contact with patient with respiratory illness (N95 respirator if performing aerosol-generating procedure or patient has suspected or confirmed infectious TB.).
- Wear surgical mask with a visor or glasses if at risk of splashes.

Get vaccinated:

- Get vaccinated against hepatitis B (if not yet done) and yearly against influenza.
- Ensure COVID-19 and pertussis vaccinations are up to date.

Know your HIV status:

- Test for HIV ➤ 110. ART and TPT can decrease the risk of TB.
- If HIV positive, you are entitled to work in an area of the facility where exposure to TB is limited.

Protect your facility

Clean the facility:

- Clean frequently touched surfaces (door handles, telephones, keyboards) daily with soap and water.
- Disinfect surfaces contaminated with blood/secretions with 70% alcohol or chlorine-based disinfectant.

Ensure adequate ventilation:

- Leave windows and doors open when possible and use fans to increase air exchange.

Organise waiting areas:

- Prevent overcrowding in waiting areas.
- Fast track patients with suspected respiratory infections, TB or acute gastroenteritis.

Manage sharps safely:

- Ensure sharps bins are easily accessible and regularly replaced.

Manage infection control in the facility:

- Appoint an infection control officer for the facility to coordinate and monitor infection control policies.

Manage possible occupational exposure promptly

Reduce TB risk

Identify patients with possible TB promptly:

- The patient with cough ≥ 2 weeks is has possible TB.
- Separate patients with possible TB from others in the facility.
- Educate about cough hygiene and give face mask/tissues to cover mouth/nose to protect others.

Diagnose TB rapidly:

- Fast track TB workup and start treatment as soon as diagnosed.

Protect yourself from TB:

- Wear an N95 respirator (not a surgical mask) if in contact with a patient with infectious TB.

Screen and test yourself for TB every 6 - 12 months:

- Screen and test for TB according to your facility policy. If TB test negative and depending on your risk profile, discuss TB preventive treatment (TPT) with your occupational health practitioner.

Reduce risk of respiratory infections (including pertussis, influenza and COVID-19)

- Before managing a patient with suspected or confirmed respiratory infection, wear appropriate PPE.
- Wash hands with soap and water. Wear a surgical mask over mouth and nose during procedures.
- Encourage patient to wear face mask, cover mouth/nose with a tissue when coughing/sneezing, to dispose of used tissues correctly and to wash hands regularly with soap/water.
- Advise patient to avoid close contact with others.